



OFFICIAL

Zuclopenthixol Acetate (Clopixol Acuphase®) Guideline

1. Purpose

To provide clear and concise guidance to clinical staff on the prescribing and administration of zuclopenthixol acetate (Clopixol Acuphase®) and the associated baseline assessments and monitoring requirements.

2. Introduction

Clopixol Acuphase® is an intermediate- acting intramuscular injection, indicated in managing acute psychosis or mania in adults. Maximum levels of zuclopenthixol are typically achieved in the serum 24–36 hours following the administration of a dose of Clopixol Acuphase®.

3. Contraindications and precautions

Table 1 Contraindications and precautions to prescribing Clopixol Acuphase®

Contraindications	Precautions (consider the following when prescribing Clopixol Acuphase®)
Known allergies to zuclopenthixol or thioxanthene antipsychotics (e.g. flupenthixol) Check if patient also has allergies to phenothiazine's (e.g. chlorpromazine) as cross-sensitivity can occur	Potential exacerbation of Parkinson's disease
Known allergies to the excipients within the formulation	Previous dystonic reaction / extrapyramidal side effects (EPSE) to zuclopenthixol / antipsychotics
Acute intoxication (e.g. alcohol or opioids)	Reduced seizure threshold
Evidence of central nervous system (CNS) or respiratory depression	Respiratory depression can occur; risk of which is amplified if patient is intoxicated, prescribed other medications that can cause sedation or affect respiratory function (e.g. benzodiazepines, opioids) or has other co- morbidities that can impede respiration (e.g. advanced chronic obstructive pulmonary disease, morbid obesity, obstructive sleep apnoea)
Circulatory collapse	Risk of arrhythmia e.g. genetic predisposition, hypokalaemia, hypomagnesaemia, concomitant

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Contraindications	Precautions (consider the following when prescribing Clopixol Acuphase®)
	medications which may prolong QTc interval, and patients with a history of cardiovascular disorders
Agranulocytosis or blood dyscrasias	Venous thromboembolism (VTE) risk should be assessed by the prescriber and documented on the relevant section of the WA Hospital Medication Chart (AKMR170B). Antipsychotics can cause thromboembolic events, so mechanical and/or pharmacological VTE prophylaxis may need to be prescribed, if clinically appropriate.
Phaeochromocytoma	History of neuroleptic malignant syndrome (NMS)
Patients who accept oral medication	Hepatic or renal impairment – dose reduction is required
Patients who are unconscious	
Pregnancy	Avoid in women of childbearing potential unless benefits outweigh potential risk to foetus if patient becomes pregnant.

4. Prescribing

Clopixol Acuphase® must be prescribed on the zuclopenthixol acetate (Clopixol Acuphase®) Chart (AKMR170.7) – see [Appendix 1](#).

Prescription alert

The usual recommended adult dose of Clopixol Acuphase® is **100mg** (up to 150mg in those who have shown previous tolerance) to be administered **every 48–72 hours** as necessary, up to a **maximum of 400mg or four doses** (whichever occurring first).

The use of Clopixol Acuphase® should not extend beyond **14 days**. Smaller doses (50mg) should be considered for special populations like the elderly or psychotropic-naïve as per the Consultant's discretion.

- The treating (or on- call) consultant must approve all orders for Clopixol Acuphase®. The medical officer must sign the relevant section on the [AKMR170.7 Chart](#) when prescribing zuclopenthixol acetate to indicate that consultant approval has been sought.
- Pharmacist to be verbally notified during working hours of doses given.
- Clopixol Acuphase® should not be prescribed in patients who are medically compromised (e.g. evidence of over sedation or respiratory distress/compromise). Prescribers should indicate that the patient's baseline physiological observations (sedation, blood pressure, pulse, oxygen saturation, respiratory rate and body temperature) have been assessed by signing the relevant column on the [AKMR170.7 Chart](#).

- Every effort should be made to obtain a baseline ECG. Where available, the prescriber must review the ECG before prescribing zuclopenthixol acetate. Zuclopenthixol can also cause QTc prolongation, so the risks and benefits of treatment should be carefully considered when prescribing concomitant medications that can also have this effect (e.g. other antipsychotics, methadone, and macrolide antibiotics). Where indicated, the prescriber must specify when a follow up ECG is to be performed.
- Each time a dose of zuclopenthixol acetate is ordered, the prescriber must review the patient's concurrently prescribed medications and existing Agitation and Arousal Medication Chart (MR170.2). In particular, consideration should be made about other antipsychotics, benzodiazepines and/or sedating medications (e.g. opioids) that are concomitantly prescribed, due to the increased risk of CNS/respiratory depression.
- If intramuscular (IM) or intravenous (IV) injections of short acting antipsychotics or sedative drugs have been given to manage agitation and arousal, allow a minimum of 60 minutes after IM and 15 minutes after IV administration for assessment of response and/or side effects of the medications before considering Clopixol Acuphase®.
- All sedatives or antipsychotics with a PRN dose frequency or on the Agitation and Arousal Medication Chart (MR170.2) must be withheld for 24 hours when patients are receiving Clopixol Acuphase®.
- Particular care should be exercised with methadone, as it is not only sedating but can also prolong the QTc interval. Where methadone is prescribed as part of the Community Program for Opioid Pharmacotherapy (CPOP), Next Step should be contacted when changes to methadone therapy are proposed. After hours, the Clinical Advisory Service (CAS) is available 24 hours to provide telephone support (contact number: (08) 9442 5042).
- The prescriber must document in the patient's progress notes that they have reviewed the above points and any additional instructions that may apply in the 72 hours post zuclopenthixol acetate administration (e.g. whether certain medications need to be withheld). In the instance where there is no documentation evident, nursing staff must contact the prescriber (or duty medical officer/ on-call Consultant, if after hours) to review the above before any antipsychotics, benzodiazepines and/or sedating medications can be given within this specified period.
- Anticholinergics, such as benztropine 1-2mg PO or IM injection up to three times daily when required, should be prescribed on the 'PRN AS REQUIRED MEDICATIONS' section of the WA Hospital Medication Chart for treatment of EPSE.
- Where possible, the patient (or their carer) should be provided written and verbal information regarding their treatment.
- Clopixol Acuphase® is to be administered **intramuscularly** into the gluteal muscle.
- Clopixol Acuphase® and Clopixol Depot® may be mixed within the same syringe and administered simultaneously.

Look-alike Sound-alike (LASA) alert

Do not confuse with Zuclopenthixol Decanoate (Clopixol Depot) 200mg/ mL IM injection (similar looking packaging).

5. Patient monitoring

5.1 Physiological observations

Table 2 Schedule for physiological observations post Clopixol Acuphase® administration

Schedule for physiological observations post Clopixol Acuphase® administration			
15 minutes	3 hours	16 hours	36 hours
30 minutes	4 hours	20 hours	40 hours
45 minutes	6 hours	24 hours	44 hours
60 minutes	8 hours	28 hours	48 hours
2 hours	12 hours	32 hours	

- Monitor the patient's physiological observations (sedation, blood pressure, heart rate, oxygen saturation, respiratory rate and body temperature) at 15-minute intervals in the initial hour after administration of Clopixol Acuphase®.
- Continue to monitor the above parameters on an hourly basis for the next three (3) hours, then monitor every two (2) hours for another four (4) hours. Thereafter, the above physiological observations can be checked on a four-hourly basis until 48 hours has lapsed since the dose of Clopixol Acuphase® was given.
- The above is a guide only and physiological observations should be performed more frequently as per the clinician's discretion, or as clinically indicated. This may be particularly relevant at 12 hours post Clopixol Acuphase® (when sedation is at its most significant) or at 24–36 hours post dose (corresponding with peak serum levels). Where the Medical Officer has not prescribed an alternative monitoring regimen, physiological observations should be monitored as per [Table 2](#) (as a minimum).
- All physiological observations must be recorded on the Adult Observation and Response Chart AKMR140.3.

5.2 Sedation

- Sedation may be seen within two (2) hours after injection, peaks after 8–12 hours and may last up to three (3) days. Care should therefore be exercised if additional doses or concurrent antipsychotics or other medications with CNS/respiratory depressant effects (e.g. benzodiazepines, opioids) are prescribed.
- When patients are asleep, at a minimum, their respiratory function must be monitored and documented on the Safe and Supportive Observation Chart AKMR147.X, in line with the frequency outlined in [Table 2](#) or as otherwise specified by the Medical Officer, as follows:
 - Respiratory rate
 - Rise and fall (depth) of chest
 - The presence of any sounds suggestive of laboured, obstructed or impaired breathing (e.g. wheezing, snoring)
- The parameters for escalation of care on the Adult Observation and Response Chart AKMR140.3 should be followed.
- If MET criteria are achieved, then a Code Blue medical emergency should be activated.

5.3 Extrapyramidal Side Effects

- Notify the team and treat immediately if EPSE occurs and document the nature of the EPSE e.g. akathisia, dystonia.
- Benztropine (as per [Section 4](#) above) can be administered when required.
- Anticholinergic drugs should not be given routinely for prophylaxis, particularly in older patients.

6. Adverse effects

The following are acknowledged as common adverse effects following post Clopixol Acuphase® administration:

- Sedation
- Dizziness
- EPSE (e.g. acute dystonia, akathisia, parkinsonism, tardive dyskinesia)
- Orthostatic hypotension
- Tachycardia
- Dry mouth.

7. Policy context

The following documents are required to give effect to this policy:

- Legislation
 - *Medicines and Poisons Act 2014 (WA)*
 - *Medicines and Poisons Regulations 2016 (WA)*
- HDWA
 - [Medication Review Policy](#)
 - [Medication Chart Policy](#)

8. Supporting information

The following documents support the implementation of this policy:

- [Recognition and Response to Acute Deterioration Policy](#)
- [Medication Policy](#)
- [Medication Management Procedure](#)
- [Safe and Supportive Observations Procedure](#)

9. Compliance, monitoring and evaluation

Compliance with this guideline will be monitored by the Mental Health Safety and Quality Committee.

10. Relevant standards

National Safety and Quality Health Service Standards (second edition):

- Standard 4 – Medication Safety - Action 4.11, 4.13 and 4.15
- Standard 8 – Recognising and responding to acute deterioration - Action 8.4

National Standards for Mental Health Services (2010):

- Standard 6 – Criterion 6.9: Consumers are provided with current and accurate information on the care being delivered.

11. References

1. MIMS Australia. (2019). Clopixol. In MIMS Online. Retrieved from <http://www.mimsonline.com.au>
2. Australian Medicines Handbook 2020 (online). Adelaide: Australian Medicines Handbook Pty Ltd; 2020 January. Available from: <https://amhonline.amh.net.au/>

12. Document owner

Service Director Mental Health

Enquiries relating to this guideline may be directed to:

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13. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency.

Version	Effective from	Effective to	Amendment(s)
V3	15 04 2026	15 04 2029	Full review, minor updates done and links updated.

14. Authorisation

This guideline has been authorised and issued by:

Authorised by	Dene olive – A/Service Director Mental Health
Authorisation date	13 04 2026
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Appendix 1 – Zuclopenthixol acetate (Clopixol Acuphase®) Chart

[Zuclopenthixol Acetate Chart AKMR 170.7.pdf](#)